City General Hospital — Department of Internal Medicine

SYNTHETIC TEST RECORD — NOT A REAL PATIENT — FOR SOFTWARE QA ONLY — FOLLOW-UP VISIT

Patient Name:	Priya Sharma	**Patient ID:**	PT-2026-002
Age / Gender:	45 years / Female	**Report Date:**	22-08-2026
Referring Physician:	Dr. Anil Mehta	**Department:**	Endocrinology

CHIEF COMPLAINT

Follow-up visit, 2 weeks after admission for uncontrolled Type 2 Diabetes Mellitus and suspected DKA. Patient reports improved energy levels and reduced thirst. Some residual fatigue remains.

DIAGNOSIS

1. Type 2 Diabetes Mellitus, improving glycemic control
2. Resolved Diabetic Ketoacidosis
3. Stage 2 Hypertension, ongoing
4. Diabetic Nephropathy — renal function monitoring, worsening trend noted

CURRENT MEDICATIONS

1. Insulin Glargine 10 units — once daily, subcutaneous (newly started)
2. Glimepiride 2 mg — once daily, oral, morning
3. Ranitidine 150 mg — twice daily, oral, for gastritis
4. Amlodipine 5 mg — once daily, oral, for hypertension

— Metformin discontinued this visit due to renal function concerns

LABORATORY RESULTS

Test	Result	Unit
Fasting Blood Glucose	210	mg/dL
HbA1c	9.8	%
Serum Potassium	5.2	mEq/L
Serum Sodium	132	mEq/L
Serum Creatinine	2.6	mg/dL
Hemoglobin	8.9	g/dL
Serum Cholesterol Total	228	mg/dL

CLINICAL NOTES

Glycemic control trending toward improvement following initiation of insulin therapy and discontinuation of metformin. However, renal function has WORSENED since last visit (creatinine 2.1 to 2.6 mg/dL); nephrology referral placed. Hemoglobin has also declined further (9.2 to 8.9 g/dL), consistent with anemia of chronic kidney disease. Potassium improved from critical range but remains above normal.

Continue close monitoring.

— End of report — Dr. Anil Mehta, MD (Internal Medicine), City General Hospital